

Positionality Statement

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Living document — revised as self-awareness deepens



Positionality is a simple idea with a clunky name: where you stand shapes what you see. Every practitioner brings a social location — a set of identities, experiences, and histories — into the room with them, whether they name it or not. This is mine, named as honestly as I can manage. It's the foundation of how I understand the people I work with, and it's the reason I trust my own judgment in rooms where my training alone wouldn't be enough.

Identity Snapshot

I occupy the following identity locations simultaneously:

- Black man in America
- Gay man — publicly out, with a history of concealing sexual orientation in professional and clinical settings due to perceived safety risks
- Person in recovery from substance use disorder — with significant lived experience across the behavioral health system
- Adult child of a family system affected by intergenerational addiction and poverty
- Person of Christian faith navigating the tension between religious community and LGBTQ+ identity
- Returning adult learner and first-generation college completer
- Behavioral health professional — currently employed as a Case Manager and Certified Recovery Specialist

No single identity on this list operates in isolation. Intersectionality isn't an academic framework I studied and adopted — it's the architecture of my actual life.

Where I Stand

An honest account of positionality can't stop at marginalization. It has to include privilege too — the things I've never had to fight for, which shape what I notice and what I miss just as much as the things I've survived.

What I've Lived Through

Race: As a Black man, I have navigated racist systems across healthcare, education, housing, and employment. The racial disparities in addiction treatment access and behavioral health outcomes are not statistics I encountered in a textbook — they are conditions I have lived inside.

Sexual Orientation: I have experienced homophobia as a structural force that directly shaped my care-seeking behavior. For years, I presented as straight in treatment settings because full disclosure felt unsafe. This is not merely a personal story — it is a case study in how homophobia functions as a systemic barrier to affirming, effective care.

Substance Use History: People in recovery from substance use disorder navigate ongoing stigma in employment, healthcare, and social contexts. My history includes more than a decade of engagement with behavioral health systems that delivered sequential rather than integrated treatment — a structural failure, not a personal one.

Economic Marginalization: My biography includes periods of homelessness and poverty. I have navigated public assistance, vocational rehabilitation, and underpaid human services employment. I understand economic injustice from the inside.

What I Haven't

I also carry privileges that shape what I assume and what I may not see:

Gender: As a man, I benefit from male privilege in professional contexts, even while navigating other forms of marginalization. Gender-based violence, the wage gap, and reproductive justice are areas where my direct experiential understanding is limited.

Citizenship and Documentation: I am a U.S.-born citizen. The experiences of undocumented immigrants, asylum seekers, and people navigating immigration enforcement are not part of my direct experience, and I must approach those populations with active humility.

Disability Status: I do not identify as having a physical disability. I may carry unconscious assumptions about ability that require ongoing examination.

English Fluency: English is my primary language. I have not navigated systems in a language other than my native tongue — a form of access that is often invisible to those who hold it.

Why This Shapes My Work

Social and Racial Justice: My experience as a Black man in American systems provides first-person evidence of how racial inequity operates at structural levels — not just in interpersonal bias, but in policy design, institutional gatekeeping, and differential outcomes.

Economic Justice: I have navigated poverty, public benefits systems, and under-resourced communities. My advocacy for economic justice is grounded in the experience of financial precarity, not observation of it.

Human Rights: Health equity, LGBTQ+ rights, and freedom from discrimination in healthcare settings are human rights issues I engage professionally through my behavioral health work — including the barriers stigma creates to prevention, treatment access, and affirming care.

Intersectionality in Practice: The populations I'm most equipped to serve span several overlapping communities: Black men carrying trauma, addiction, and the pressure to perform strength instead of ask for help; people in recovery who've experienced treatment as a series of disconnected interventions rather than whole-person care; and LGBTQ+ people navigating faith communities that weren't always safe. These are exactly the populations whose needs get missed by practitioners who treat identity as a single axis instead of an intersection. This isn't a framework I apply from the outside. It's the calling that shaped my career choice.

Boundaries I Hold

Proximity to client experience: My lived experience is a clinical asset that requires active boundary management. I use personal experience to inform practice — not to drive unsanctioned self-disclosure or over-identification with the people I work with.

Risk of proximity bias: My strongest empathic connection is with populations that mirror my own experience. I have to actively cultivate the same depth of empathy for populations whose oppressions operate differently from mine — including women, people with disabilities, undocumented individuals, and people whose cultural frameworks differ significantly from my own.

The burden of representation: In professional and academic settings, I'm sometimes one of very few people present who hold my particular combination of identities. I'm not obligated to serve as an educator for every question my presence raises. My participation in these spaces is purposeful and boundaried — it serves my own growth and the group's, not personal processing on someone else's time.

Power differential as a practitioner: As someone training for clinical licensure, I'm preparing to hold institutional authority over clients — including clients who may share my history. The shift from having once needed help to being the one offering it carries its own ethical weight, and I don't take that lightly.

What I Commit To

- Engage honestly with self-reflection while maintaining appropriate boundaries around what I disclose and to whom.
- Actively examine the dimensions of my own privilege alongside my experiences of marginalization.
- Approach people whose experiences differ from mine with deliberate humility and a genuine commitment to learning.
- Stay critically self-aware — recognizing when my own values, assumptions, or reactions are shaping how I interpret a situation.

- Treat this reflection as preparation for culturally humble, justice-informed practice — not as a substitute for my own therapy.

Anchor Scripture

Romans 8:38-39

This scripture anchors my conviction that every human being — regardless of identity, history, or social location — holds inherent dignity. That conviction is the foundation of my commitment to social justice practice.

“For I am convinced that neither death nor life, neither angels nor demons, neither the present nor the future, nor any powers, neither height nor depth, nor anything else in all creation, will be able to separate us from the love of God that is in Christ Jesus our Lord.”